Anti-Amyloid Infusion Program Standard Operating Procedure

SYNTHETIC DOCUMENT. Fabricated for a demo. Northlake Health is fictional; aggregate planning figures only, no patient-level data.

Owner: Ambulatory Infusion Operations with the Memory Care Clinical Council. **Effective:** on Option A approval. This is an operational planning document, not clinical guidance. Treatment decisions belong to the treating clinician and current FDA labeling.

1. Purpose and scope

This procedure covers referral-to-chair operations for FDA-approved anti-amyloid antibodies at the 3 dedicated West Campus infusion chairs. Lecanemab is infused every two weeks and donanemab every four weeks, per product labeling.

2. Eligibility workflow (operational checklist)

The nurse navigator owns the checklist in the EHR. A patient is ready for a first chair booking only when every item is complete:

- Clinical evaluation completed under the draft clinical pathway (early symptomatic Alzheimer's disease).
- Amyloid pathology confirmed by amyloid PET or CSF testing.
- APOE e4 testing and genetic counseling documented.
- Recent baseline brain MRI read and on file.
- Anticoagulant risk discussion documented when applicable.
- Prior authorization approved, where the payer requires it.
- Treating clinician registered with the qualifying CMS registry, and the patient enrolled for registry data submission.

Staffing reference: 1 nurse navigator per 60 to 70 active patients.

3. ARIA safety MRI schedule

Safety MRIs follow the intervals in each product label, plus unscheduled scans when symptoms appear. Radiology books ARIA-protocol scans into the 6 protected 3T slots per week and uses the standard ARIA protocol with SWI and FLAIR sequences.

- Read target: 24 hours for ARIA-protocol scans.
- The safety MRI report must be posted at least 2 business days before the next scheduled infusion. If it is not, the infusion nurse reschedules the chair and notifies the navigator.
- When a report shows ARIA, dosing is held and the prescriber is notified the same day under the hold-and-review protocol.
- Year-one planning volume: about 880 added MRIs, including about 630 safety-monitoring scans.

4. Documentation for payers

Attach monitoring MRI reports to every re-authorization request. Missing MRI documentation was the top denial reason for peer systems.

5. Chair scheduling

- Lecanemab visits book a 2 chair-hour slot; donanemab visits book a 2.5 chair-hour slot. Both include post-infusion observation.
- The infusion center operates 12 hours a day. Anti-amyloid slots start at 7:00 AM, and the last anti-amyloid slot of the day starts no later than 3:30 PM so observation ends well before closing.
- Year-one planning demand is about 5,900 chair-hours, the equivalent of 2 chairs at full use. The third dedicated chair absorbs growth and no-shows.
- Reassess capacity when the dedicated chairs reach 80% utilization.

6. Missed visits

The navigator contacts the patient and care partner within 1 business day of a missed infusion and rebooks within the labeled dosing window. Transportation barriers are referred to Care Management.